



Detainee Death Review Report

Charles DANIEL

Date of Death: March 7, 2024

Alien File Number: 022 745 951

Joint Integrity Case Management System (JICMS) Number: (b)(7)(E)

August 22, 2024

U.S. Immigration and Customs Enforcement
Office of Professional Responsibility

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DETAILS OF REVIEW

From April 2 to 4, 2024, U.S. Immigration and Customs Enforcement (ICE), Office of Professional Responsibility (OPR) External Reviews and Analysis Unit (ERAU) staff reviewed the facts and circumstances surrounding Charles DANIEL's (DANIEL) detention and death while in ICE custody. ERAU was assisted in its review by contract subject-matter experts in correctional healthcare and security who are employed by Creative Corrections, a national management consulting firm. As part of its review, ERAU reviewed immigration, medical, and detention records pertaining to DANIEL's time in custody, in addition to conducting interviews of the individuals employed by the Northwest ICE Processing Center (NWIPC) in Tacoma, Washington (WA).

DANIEL, a 61-year-old, Trinidadian male, was detained at NWIPC from April 2, 2020, until his death on March 7, 2024. The State of Washington Certificate of Death documented DANIEL's cause of death¹ as hypertensive² and atherosclerotic cardiovascular disease.³

FACILITY PROFILE

NWIPC is owned and operated by the GEO Group, Inc. (GEO). The local ICE Enforcement and Removal Operations (ERO) field office in Seattle, WA (ERO Seattle) oversees the management of NWIPC. ICE houses male and female detainees at NWIPC for periods longer than 72 hours, pursuant to a contractual agreement requiring the facility to comply with the ICE Performance-Based National Detention Standards (PBNDS) 2011 (Revised 2016).⁴ GEO personnel provide security and ICE Health Service Corps (IHSC) provides medical services at NWIPC. At the time of DANIEL's death, the ICE detainee average daily population was (b)(7)(E) and NWIPC housed (b)(7)(E) detainees.⁵

IMMIGRATION AND CRIMINAL HISTORY

On July 11, 2000, DANIEL entered the United States at Miami, Florida as a nonimmigrant visitor, with authorization to remain in the United States until January 10, 2001.

On May 19, 2003, the Seattle Police Department in Seattle, WA arrested DANIEL for murder in the first degree.⁶ On October 29, 2003, the King County Superior Court (KCSC) in Seattle, WA convicted DANIEL of murder in the second degree and sentenced him to 220 months confinement in the Washington State Penitentiary.⁷

¹ See **Exhibit 1**: State of Washington Certificate of Death, issued June 26, 2024.

² Hypertensive cardiovascular disease refers to heart problems that occur due to high blood pressure.

³ Atherosclerotic cardiovascular disease occurs when the blood vessels that carry oxygen and nutrients from the heart to the rest of the body become thick and stiff, sometimes restricting blood flow to the organs and tissues.

⁴ See ERO Facility List Report, dated March 11, 2024.

⁵ *Ibid.*

⁶ See Record of Deportable/Inadmissible Alien (Form I-213), dated April 20, 2020.

⁷ *Ibid.*

On March 31, 2020, the Washington State Penitentiary released DANIEL into the custody of ERO Seattle, who held him at the Kittitas County Jail in Kittitas, WA, pending his transfer to NWIPC.⁸

On April 2, 2020, ERO Seattle served DANIEL with a Notice to Appear (NTA), Form 1-862, charging him with removability pursuant to Section 237(a)(1)(B), for remaining in the United States for a time longer than permitted; Section 237(a)(2)(A)(iii), for being convicted of an aggravated felony; and Section 237(a)(2)(A)(i), for being convicted of a crime involving moral turpitude.⁹ ERO Seattle transferred DANIEL to the NWIPC, pending a hearing before an immigration judge.

CHRONOLOGY OF EVENTS¹⁰

April 2, 2020

At 4:30 p.m., DANIEL arrived at NWIPC.¹¹ (b)(6),(b)(7)(C),(b)(7)(F) and (b)(6),(b)(7)(C),(b)(7)(F) NWIPC detention officers (DOs), completed his intake processes.¹² (b)(6),(b)(7)(C),(b)(7)(F) NWIPC classification coordinator, classified DANIEL as high security.¹³

At 7:34 p.m., (b)(6),(b)(7)(C),(b)(7)(F) registered nurse (RN), completed DANIEL's medical intake screening, documenting the following:¹⁴

- DANIEL presented with high blood pressure at 135/78,¹⁵ while all other vital signs were within normal limits;
- DANIEL reported past medical issues with kidney stones and mild hypertension;¹⁶
- DANIEL denied symptoms of tuberculosis (TB);¹⁷
- DANIEL reported he had received counseling and medication for mental health problems;
- DANIEL reported he was taking fluoxetine,¹⁸ 10 milligrams (mg), 1 capsule by mouth every 3 days; and

⁸ See (b)(7)(E) Detention History, dated March 31, 2020.

⁹ See Record of Deportable/Inadmissible Alien (Form I-213), dated April 20, 2020.

¹⁰ Given the length of DANIEL's detention in ICE custody (1,435 days), events within this report are limited to key events.

¹¹ See Subject Profile, dated April 2, 2020.

¹² See Intake documents, dated April 2, 2020.

¹³ See ICE Custody Classification Worksheet, dated April 2, 2020.

¹⁴ See Clinical entry - Intake, dated April 2, 2020.

¹⁵ Per American Heart Association (AHA) guidelines, normal blood pressure for adults is a systolic blood pressure <120, and a diastolic blood pressure of <80.

Although DANIEL's blood pressure was higher than normal per AHA Guidelines, IHSC's threshold programmed in eCW is higher than AHA's threshold. The eCW system flags readings as abnormal when one or both measurements exceed 140/90. For this report, blood pressure readings are noted as normal unless flagged by eCW or addressed by a practitioner in the clinical entry.

¹⁶ Hypertension is blood pressure exceeding the normal range.

¹⁷ According to the medical transfer summary, DANIEL refused a TB skin test. TB screening did not identify any signs or symptoms of TB.

See Medical Transfer Summary, dated April 2, 2020.

¹⁸ Fluoxetine is a medication used to treat depression.

- DANIEL denied suicidal or homicidal ideations.

DANIEL signed a consent form to receive medical and mental health care.¹⁹ RN (b)(6),(b)(7)(C),(b)(7)(F) offered DANIEL an influenza vaccination, which he refused.²⁰

RN (b)(6),(b)(7)(C),(b)(7)(F) referred DANIEL to a medical provider, requested a mental health assessment, and notified (b)(6),(b)(7)(C),(b)(7)(F) Doctor of Psychology (Psy.D.), that Washington State Prison medical staff diagnosed DANIEL with depressive disorder and persecutory delusions.²¹ Dr. (b)(6),(b)(7)(C),(b)(7)(F) referred DANIEL to (b)(6),(b)(7)(C),(b)(7)(F) Psy.D., who scheduled him for a mental health appointment the following day. RN (b)(6),(b)(7)(C),(b)(7)(F) educated DANIEL on how to access medical and mental health care at NWIPC and cleared DANIEL for housing in the COVID-19 quarantine housing area.²²

April 3, 2020

(b)(6),(b)(7)(C),(b)(7)(F) physician assistant (PA), completed DANIEL's initial physical examination and health appraisal,²³ noting the following:

- DANIEL presented with high blood pressure at 172/87,²⁴ while all other vital signs were within normal limits;
- DANIEL reported the removal of an abdominal lipoma²⁵ in January 1997;
- DANIEL reported he received psychiatric counseling and medication for depression while in prison.
- DANIEL's speech was rapid, and his thought content was tangential;²⁶
- DANIEL reported he was being electronically harassed and that people put spirits in him. He stated he heard voices and saw swirling air coming through the vents, commenting officers heard and saw it also;
- DANIEL denied thoughts of suicidal or homicidal ideations; and
- DANIEL reported swelling of his legs and stated he used elastic bandage wraps to help with the swelling.

PA (b)(6),(b)(7)(C),(b)(7)(F) diagnosed DANIEL with bilateral edema²⁷ and offered DANIEL thiazide²⁸ to treat his edema, but DANIEL declined new medications. PA (b)(6),(b)(7)(C),(b)(7)(F) recommended lab tests, but DANIEL refused the tests. PA (b)(6),(b)(7)(C),(b)(7)(F) ordered fluoxetine (10 mg), 1 tablet daily. PA (b)(6),(b)(7)(C),(b)(7)(F) educated DANIEL on a healthy lifestyle, dental and personal hygiene. PA (b)(6),(b)(7)(C),(b)(7)(F) ordered daily

¹⁹ See IHSC Medical Consent Form, signed April 2, 2020.

²⁰ See IHSC Refusal Form, dated April 2, 2020.

²¹ Depression is a mood disorder disorders marked by loss of interest or pleasure in living. A person with persecutory type depression believes a person or group wants to hurt them.

²² See GEO Track housing history, dated April 2, 2020.

²³ See Clinical entry - PE - Initial, dated April 3, 2020.

²⁴ Blood pressure of 172/87 falls within AHA parameters for Stage 2 hypertension. Stage 2 hypertension is when blood pressure consistently ranges from 140 to 179 systolic and 90 to 119 diastolic. At this stage of high blood pressure, health care professionals should prescribe blood pressure medication and lifestyle changes.

²⁵ Lipoma is a slow-growing, fatty lump that is usually harmless.

²⁶ Tangential means to speak about topics unrelated to the main topic of discussion.

²⁷ Edema is a condition in which body tissues contain excessive amounts of fluid in interstitial spaces.

²⁸ Thiazide is a medication that increases urine flow and reduces fluid volume and blood pressure.

blood pressure checks to monitor DANIEL's high blood pressure and noted DANIEL had already been referred to mental health.²⁹ ERAU reviewed the record and found PA (b)(6),(b)(7)(C),(b)(7)(F) did not obtain an informed consent form for psychotropic medication from DANIEL nor discuss the possible side effects of the fluoxetine with him.³⁰

On the same day, Dr. (b)(6),(b)(7)(C),(b)(7)(F) conducted a mental health assessment of DANIEL, noting the following:³¹

- DANIEL reported previous mental health treatment while in prison, stating he was involuntarily admitted to the Special Offender Unit;³²
- DANIEL reported taking fluoxetine for depression since 2018;
- DANIEL denied anxiety but reported he was worried about his case; and
- DANIEL denied auditory and visual hallucinations, but remarked, "It doesn't have to be heard; its [*sic*] auditory perception. It's like electronic harassment." DANIEL further described paranoid delusions, stating the electronic harassment comes through the air vents.

Dr. (b)(6),(b)(7)(C),(b)(7)(F) deferred a mental health diagnosis and noted her plan was for DANIEL to remain in general population, follow-up with a provider in 4 to 5 weeks, or sooner if necessary, and refer to the psychiatrist for medication management. Dr. (b)(6),(b)(7)(C),(b)(7)(F) educated DANIEL on how to access emergency services if he needed them and discussed hygiene, relaxation, and stress management skills. DANIEL verbalized understanding and agreed with the treatment plan.

April 6, 2020

(b)(6),(b)(7)(C),(b)(7)(F) advanced registered nurse practitioner (ARNP), conducted a follow-up with DANIEL for his hypertension,³³ noting the following:

- DANIEL presented with high blood pressure at 160/97,³⁴ while all other vital signs were within normal limits;
- DANIEL's blood pressure was classified as stage 2 hypertension with poor control according to IHSC's clinical guidelines for hypertension;³⁵ and
- DANIEL's blood pressure checks continued to show high blood pressure, and medical staff could only obtain electronic checks³⁶ because DANIEL refused manual readings when attempted to confirm the electronic readings.

²⁹ See IHSC Medical/Psychiatric Alert form, dated April 3, 2020.

A Medical/Psychiatric alert prompts medical clearance prior to transfer or removal from NWIPC.

³⁰ ERAU was unable to interview to conduct an interview with PA (b)(6),(b)(7)(C),(b)(7)(F) as they no longer work for NWIPC.

³¹ See Clinical entry - MH Intake, dated April 3, 2020.

³² The Special Offender Unit is a unit that provides treatment, re-entry services, and program planning for mentally ill offenders within the Washington State Department of Corrections.

³³ See Clinical entry - HTN, dated April 6, 2020.

³⁴ Blood pressure of 160/97 falls within AHA parameters for Stage 2 hypertension.

³⁵ See IHSC Provider Clinical Guidelines: Hypertension, dated May 25, 2017.

³⁶ Electronic blood pressure checks are digital blood pressure monitors that are often used on the wrist and provide one's blood pressure.

ARNP [REDACTED] discussed DANIEL's blood pressure readings with him and the importance of pharmacological treatment and diagnosed him with essential hypertension.³⁷ DANIEL remarked there were external forces and electrical impulses causing his elevated blood pressure.³⁸ ARNP [REDACTED] attempted to obtain DANIEL's labs, but he refused. ARNP [REDACTED] offered DANIEL several medication options, but he would only consider chlorthalidone,³⁹ (25 mg), one tablet daily, which she prescribed. Although DANIEL agreed to take chlorthalidone, he refused to take the medication from April 9 through 13, 2020, remarking the medication worsened his condition.⁴⁰

April 14, 2020

ARNP [REDACTED] conducted a follow-up with DANIEL to discuss his concerns with the medication.⁴¹ ARNP [REDACTED] noted DANIEL's blood pressure had improved but remained high at 150/82,⁴² while all other vital signs were within normal limits. DANIEL stated he did not need to be treated pharmacologically for high blood pressure and when he took the medication for 3 days, his blood pressure increased. ERAU's review of DANIEL's medical record shows his blood pressure improved following his use of chlorthalidone for 3 days.⁴³ ARNP [REDACTED] discussed the risks of not treating hypertension, including organ damage and possible death. DANIEL voiced understanding, and ARNP [REDACTED] stopped chlorthalidone per DANIEL's request.

April 16, 2020

DANIEL requested to be moved to protective custody, reporting it was for the fear and safety of others and he believed the government was electronically monitoring him.⁴⁴ [REDACTED] NWIPC Lieutenant (LT), completed an administration segregation order, and DANIEL was moved to the Restricted Housing Unit (RHU).⁴⁵ Upon his arrival in the RHU, [REDACTED] RN, completed DANIEL's segregation assessment, noting his blood pressure was high at 138/71,⁴⁶ while all other vital signs were within normal limits.⁴⁷ DANIEL denied thoughts of harming himself or others and did not have any medical complaints. RN [REDACTED] informed DANIEL how to access medical, mental health, and dental care while in the RHU.

DANIEL remained in the RHU in protective custody until the date of this death. ERAU's review of the record and interviews with facility staff showed:

³⁷ Essential hypertension is the most common form of hypertension without an identifiable physiologic cause.

³⁸ See Clinical entry - HTN, dated April 6, 2020.

³⁹ Chlorthalidone is a diuretic which reduces the amount of water in the body by increasing the flow of urine, which helps to lower blood pressure.

⁴⁰ See IHSC Refusal Forms, dated April 9, 2020, through April 14, 2020.

⁴¹ See Clinical entry - Noncompliance with medications, dated April 14, 2020.

⁴² Blood pressure of 158/82 falls within AHA parameters for Stage 2 hypertension.

⁴³ Blood pressure checks of DANIEL following the use of chlorthalidone: April 8, 2020, 171/88; April 10, 2020, 152/76; and April 12, 2020, 157/85.

See Clinical entries - BP Check per [REDACTED] dated April 8, 10, and 12, 2020.

⁴⁴ See Administrative Segregation Order, dated April 16, 2020.

⁴⁵ See GEO Track housing history.

⁴⁶ Blood pressure of 138/71 falls within AHA parameters for Stage 1 hypertension. Stage 1 hypertension is when blood pressure consistently ranges from 130 to 139 systolic and 80 to 89 diastolic.

⁴⁷ See Clinical entry - Segregation Clearance, dated April 16, 2020.

- RHU DOs completed all required status reviews of DANIEL and conducted weekly in-person interviews with him to discuss his segregation status, noting DANIEL consistently requested to remain in protective custody because he preferred to be alone..⁴⁸
- DOs offered recreation and showers to DANIEL; however, he generally refused..⁴⁹ During an interview with ERAU (b)(6),(b)(7)(C),(b)(7)(F) RHU DO, stated he was assigned to the RHU throughout DANIEL's time in the unit and never saw DANIEL leave his cell for a shower or recreation..⁵⁰ DO (b)(6),(b)(7)(C),(b)(7)(F) further remarked that DANIEL did not like to leave his cell because he did not want staff to search or steal his belongings; specifically, his paperwork. RHU staff reported that while DANIEL refused to leave his cell for a shower, he maintained his personal hygiene by taking "birdbath showers"..⁵¹ in his cell. DANIEL accepted meals but would wash his food in the sink of his cell because he thought the food was poisoned..⁵²
- Facility medical staff completed daily rounds to assess if DANIEL had any complaints or concerns, thoughts of self-harm, and if he wanted to see medical..⁵³ and
- Facility mental health providers conducted weekly face-to-face rounds to monitor DANIEL's mental health status, identify signs of deterioration, and recommend additional treatment..⁵⁴

April 21, 2020

PA (b)(6),(b)(7)(C),(b)(7)(F) conducted a follow-up with DANIEL following his refusal to take his prescribed fluoxetine..⁵⁵ PA (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL refused the medication because he wanted fluoxetine on Mondays, Thursdays, and Saturdays rather than daily as prescribed. DANIEL stated he needed the medication to keep his clarity; also, that he believed facility staff kept the medication from him so he would not be clear for his court date next week. DANIEL further stated he could not take anything to decrease his blood pressure because he will have nothing left to fight the electronic harassment to which he is subjected to every day. DANIEL also reported the edema in his lower legs was still present but believed it had improved since he began washing his food. PA (b)(6),(b)(7)(C),(b)(7)(F) did not attempt starting any new treatment because of DANIEL's state of mind and noted he had an appointment with mental health the following day.

⁴⁸ See Administrative Detention Reviews, dated April 16, 2020, though March 7, 2024.

⁴⁹ See Detention file. See also ERAU interview with (b)(6),(b)(7)(C),(b)(7)(F) Chief of Security, (b)(6),(b)(7)(C),(b)(7)(F) Lieutenant, and (b)(6),(b)(7)(C),(b)(7)(F) DOs, on March 29, April 2 and 4, 2024.

⁵⁰ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) on March 29, 2024.

⁵¹ A birdbath shower or bird bathing is washing oneself with a washcloth or towel in the sink.

⁵² See ERAU interview with (b)(6),(b)(7)(C),(b)(7)(F) Chief of Security, (b)(6),(b)(7)(C),(b)(7)(F) Lieutenant, and (b)(6),(b)(7)(C),(b)(7)(F) DOs, on March 29, April 2 and 4, 2024.

⁵³ See RHU Medical and Mental Health Logs, dated April 16, 2020, though March 7, 2024.

⁵⁴ Ibid.

⁵⁵ See Clinical entry - SC: Medication s/e, refill, dated April 21, 2020. The MAR shows he refused fluoxetine on April 14 and 19, 2020.

April 22, 2020

Dr. (b)(6),(b)(7)(C),(b)(7)(F), psychiatrist, evaluated DANIEL,⁵⁶ noting the following:

- DANIEL reported he was diagnosed with delusional disorder and medicated with Risperidone.⁵⁷ by force many years ago for a short period of time;
- DANIEL stated he did not agree with the delusional disorder diagnosis and refused taking the medication;
- DANIEL reported he was harassed by electronic monitoring and special waves were targeting him;
- DANIEL felt his food had a special ingredient to lower his energy and he counteracted it by taking fluoxetine three times a week;
- DANIEL requested to continue the fluoxetine but wanted it only on Monday, Thursday, and Saturday; and
- DANIEL refused a base line lab/blood test, stating they may inject poison in him as they did while he was in the state prison.

Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted fluoxetine was not indicated to treat delusional thinking, but he would continue the medication since DANIEL had had taken it for 2 years. Dr. (b)(6),(b)(7)(C),(b)(7)(F) also noted DANIEL felt distressed and depressed, and continuation of fluoxetine may help with his secondary depression. Dr. (b)(6),(b)(7)(C),(b)(7)(F) prescribed fluoxetine (10 mg), 3 times a week on Monday, Thursday, and Saturday, and scheduled him for a follow-up in 4 to 6 weeks. DANIEL signed a consent form for the medication.⁵⁸

April 29, 2020

PA (b)(6),(b)(7)(C),(b)(7)(F) conducted a follow-up with DANIEL, noting the following:⁵⁹

- DANIEL presented with high blood pressure at 168/108,⁶⁰ while all other vital signs were within normal limits;
- DANIEL continued to refuse medications to control blood pressure because he believed the medications may contain poison; and
- DANIEL requested a magnet and sea salt to ward off the spirits and block electronic harassment and a sperm and fecal test to find out if the poisons were attacking him.

PA (b)(6),(b)(7)(C),(b)(7)(F) again discussed the risks of hypertension with DANIEL, including myocardial infarction, stroke, and kidney failure and informed him that sea salt and magnets could not be prescribed, and he did not meet criteria for sperm and fecal tests.

⁵⁶ See Clinical entry - Pt came to facility with psychotropic medication, dated April 22, 2020.

⁵⁷ Risperidone is a medication used to manage psychotic disorders.

⁵⁸ See IHSC consent for Psychotropic Medications for Adults form, dated April 22, 2020.

⁵⁹ See Clinical entry - SC: Multiple requests, dated April 29, 2020.

⁶⁰ Blood pressure of 168/108 falls within AHA parameters for Stage 2 hypertension.

May 1, 2020

(b)(6),(b)(7)(C),(b)(7)(F) PA, examined DANIEL pursuant to a sick call request for blood in his urine..⁶¹ PA (b)(6),(b)(7)(C),(b)(7)(F) assessed DANIEL and performed a urine dipstick test,⁶² which confirmed the presence of blood..⁶³ DANIEL expressed delusional thoughts, stating he was being tortured at NWIPC by having toxic substances being put into his body and believed sperm and fecal tests were needed to prove his case. DANIEL further remarked that there were harmful radio waves and extrinsic elements entering his body through the air vents in his cell and the food he eats. PA (b)(6),(b)(7)(C),(b)(7)(F) determined DANIEL had hematuria⁶⁴ and ordered a routine urinalysis and culture to be completed the next day..⁶⁵ DANIEL refused the urinalysis and culture test ordered by PA (b)(6),(b)(7)(C),(b)(7)(F)..^{66, 67} DANIEL also refused his scheduled follow-up appointment with PA (b)(6),(b)(7)(C),(b)(7)(F)..⁶⁸

May 12, 2020

Dr. (b)(6),(b)(7)(C),(b)(7)(F) Dr. (b)(6),(b)(7)(C),(b)(7)(F) and Dr. (b)(6),(b)(7)(C),(b)(7)(F) met with DANIEL to discuss his symptoms and changed his diagnosis to delusional disorder..⁶⁹ The following day, Dr. (b)(6),(b)(7)(C),(b)(7)(F) conducted RHU rounds and documented DANIEL was not exhibiting active symptoms and did not meet IHSC criteria for a patient with a serious mental illness (SMI)..⁷⁰

September 15, 2020

Dr. (b)(6),(b)(7)(C),(b)(7)(F) assessed DANIEL during a follow-up noting he refused fluoxetine for the last 3 weeks..⁷¹ Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL reported that unless the fluoxetine came from his personal property, he would refuse it. DANIEL further remarked he believed the facility's supply of fluoxetine was tainted and contaminated; it did not help his energy level and did not decrease inflammation. DANIEL said no one understood the level of persecution he felt and strongly believed that his activity and thoughts were monitored by the government via high frequency waves. Dr. (b)(6),(b)(7)(C),(b)(7)(F) discontinued DANIEL's fluoxetine prescription and scheduled a follow-up in 1 month or earlier if needed.

⁶¹ See Clinical entry - RHU ROUNDS, dated May 1, 2020.

Scanned Sick Call form completed by RN (b)(6),(b)(7)(C),(b)(7)(F) was not located in the medical file.

⁶² A urine dipstick test is a test that uses a special strip of paper that is dipped into a sample of urine. The result is available almost immediately.

⁶³ See Clinical entry - SC: Genitourinary blood in urine, dated May 1, 2020.

⁶⁴ Hematuria is blood in the urine.

⁶⁵ Urine culture test checks for germs that cause infections.

⁶⁶ See IHSC Refusal Form, dated May 2, 2020.

⁶⁷ From May 2020 until DANIEL's death in March 2024, NWIPC medical and mental health care staff continued their attempts to treat DANIEL for his mental health and medical issues. They noted DANIEL continued to report harassment by electronic monitoring, ingredients in his food lowered his energy, and special waves were targeting him. Throughout his detention in the RHU, DANIEL refused to accept the diagnosis of delusional disorder and refused follow-ups for his hypertension.

⁶⁸ See IHSC Refusal Form, dated May 5, 2020.

⁶⁹ See Clinical entry - Consultation with MH Treatment Team, dated May 12, 2020.

⁷⁰ See Clinical entry - MH Rounds, dated May 13, 2020.

⁷¹ See Clinical entry - Delusional Disorder, dated September 16, 2020.

October 26, 2020

Dr. [REDACTED] assessed DANIEL pursuant to a request by DANIEL to discuss the medication, citalopram.⁷² Dr. [REDACTED] noted fluoxetine was discontinued on September 15, 2020, and DANIEL was again experiencing tiredness, low energy, and an inability to focus. DANIEL stated he was stressed about an upcoming court hearing and that coping with what he believed was persecution made him depressed. Dr. [REDACTED] prescribed citalopram (10 mg), 1 tablet daily, and cholecalciferol⁷³ (2000 IU), 1 tablet daily. DANIEL signed a consent form for the citalopram.⁷⁴ Dr. [REDACTED] scheduled DANIEL for a follow-up appointment in 1 week.

November 3, 2020

Dr. [REDACTED] conducted a follow-up with DANIEL and noted he believed the cholecalciferol supplied by the pharmacy tasted funny and feared it was contaminated.⁷⁵ DANIEL reported the citalopram improved his tiredness, low energy, and inability to focus. Dr. [REDACTED] continued the citalopram, discontinued the cholecalciferol, and started a daily multivitamin tablet.

December 1, 2020

Dr. [REDACTED] attempted to conduct a follow-up with DANIEL, but he refused.⁷⁶ Dr. [REDACTED] noted DANIEL was medication compliant, continuing his prescriptions for citalopram and the multivitamin.⁷⁷

January 5, 2021

Dr. [REDACTED] conducted a follow-up with DANIEL noting he started citalopram, per his own request, but stopped taking it on December 15, 2020, because the brand of the medication changed.⁷⁸ Dr. [REDACTED] prescribed paroxetine HCl,⁷⁹ (10 mg), 1 tablet daily. Dr. [REDACTED] informed DANIEL he would not prescribe a new medication in the future when the brand changed and continued paroxetine HCl through April 2021, noting DANIEL reported he felt better with the new medication.⁸⁰

April 28, 2021

During a mental health segregation round, DANIEL reported to Dr. [REDACTED] that a staff member made remarks he interpreted as sexual harassment and he wanted to make a complaint.⁸¹ DANIEL refused to go to the medical department for a SAAP evaluation or answer questions regarding the

⁷² Citalopram, commonly known by the brand name Celexa, is a medication used to treat depression.

⁷³ Cholecalciferol is a form of Vitamin D.

⁷⁴ See Consent to Use of Serotonin Reuptake Inhibitor Antidepressant Medication form, dated October 26, 2020.

⁷⁵ See Clinical entry - "Feels stressed out", dated November 3, 2020.

⁷⁶ See IHSC Refusal Form, dated December 1, 2020.

⁷⁷ See clinical encounter titled History of Present Illness, dated December 1, 2020.

⁷⁸ See clinical encounter titled: Refused Citalopram, dated January 5, 2021.

⁷⁹ Paroxetine HCl is a medication used to treat depression.

⁸⁰ See clinical encounters, dated January 25, 2021, February 22, 2021, March 31, 2021, and April 28, 2021.

⁸¹ See MH seg rounds, dated April 28, 2021.

incident, stating he already reported the incident and did not want to leave his unit to discuss the complaint further.⁸² (b)(6),(b)(7)(C),(b)(7)(F) PA, discussed the allegation with DANIEL, during which DANIEL stated he was not physically assaulted in any way.⁸³ He said speech emanates from the vent in his room, and he believed a voice in the vent made the sexual remarks.

June 23, 2021

(b)(6),(b)(7)(C),(b)(7)(F) licensed clinical social worker (LCSW), assessed DANIEL during segregation rounds and noted an officer expressed concern about DANIEL's paranoid thoughts and delusions.⁸⁴ The officer stated DANIEL was banging on his cell door, demanding to speak with a supervisor, and accused GEO of murdering his mother and threatening his brother. LCSW (b)(6),(b)(7)(C),(b)(7)(F) assessment found DANIEL's thought process was unclear and nonlogical and he displayed signs of psychosis, which were outside his baseline behavior. DANIEL was agitated, his affect was animated, he had poor eye contact, and he demonstrated signs of paranoia. LCSW (b)(6),(b)(7)(C),(b)(7)(F) discussed DANIEL's behavior at the mental health team meeting the same day and the team determined DANIEL should be admitted to the Medical Housing Unit (MHU) for mental health observation. LCSW (b)(6),(b)(7)(C),(b)(7)(F) scheduled follow-up appointments with Dr. (b)(6),(b)(7)(C),(b)(7)(F) and Dr. (b)(6),(b)(7)(C),(b)(7)(F).

Dr. (b)(6),(b)(7)(C),(b)(7)(F) assessed DANIEL for admission to the MHU for mental health observation, noting the following:⁸⁵

- DANIEL was pleasant and cooperative and denied suicidal thoughts or audio or visual hallucinations;
- DANIEL said no one believes him and that electronic waves are monitoring his thinking;
- DANIEL reported depressed mood and feeling distressed;
- DANIEL denied sleep problems; and
- DANIEL appeared to have increased paranoid thoughts and delusions and recent delusional thoughts about GEO officers;

Dr. (b)(6),(b)(7)(C),(b)(7)(F) admitted DANIEL to the MHU for mental health observation and ordered 15-minute irregular checks by security staff and nursing checks every 8 hours.⁸⁶ Dr. (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL would be discharged from MHU when he demonstrated prosocial problem solving, stability of his mood, and safe behaviors.

Staff observed DANIEL for 2 days and found he displayed safe behaviors, reported future focus, a sense of resiliency, and a belief system that discouraged suicide. Dr. (b)(6),(b)(7)(C),(b)(7)(F) psychiatrist, discussed DANIEL's condition with the mental health treatment team, and the team concluded DANIEL could return to the RHU.⁸⁷

⁸² See IHSC Refusal Form, dated April 28, 2021.

⁸³ See clinical encounter titled: SAAP, dated April 28, 2021.

⁸⁴ See clinical encounter titled: Mental health seg, dated June 23, 2021.

⁸⁵ See clinical encounter titled: MH SRA, dated June 23, 2021.

⁸⁶ See clinical encounter titled: MHU Admit, dated June 23, 2021.

⁸⁷ See clinical encounter titled: ADM DISCHARGE FROM MHU, dated June 25, 2021.

July 19, 2021

Dr. (b)(6),(b)(7)(C),(b)(7)(F) conducted a follow-up with DANIEL noting his delusional thoughts were still present but had not changed in intensity..⁸⁸ Dr. (b)(6),(b)(7)(C),(b)(7)(F) continued paroxetine HCl and noted he wanted to explore the possibility of placement in another ICE facility that could force treatment for psychosis..⁸⁹ During an interview with ERAU, Dr. (b)(6),(b)(7)(C),(b)(7)(F) stated that he was aware that the option to transfer DANIEL was discussed, but a facility that could take him was not identified..⁹⁰

March 3, 2022

PA (b)(6),(b)(7)(C),(b)(7)(F) attempted to conduct an annual health appraisal of DANIEL, but he refused to be seen. PA (b)(6),(b)(7)(C),(b)(7)(F) was able to obtain DANIEL's vital signs and noted his blood pressure was high at 186/112..⁹¹ while all other vital signs were within normal limits..⁹² During an interview with ERAU, PA (b)(6),(b)(7)(C),(b)(7)(F) stated he did not address the blood pressure reading because DANIEL consistently refused treatment for hypertension..⁹³ PA (b)(6),(b)(7)(C),(b)(7)(F) further stated he did not treat DANIEL's hypertension as emergent because he did not have related symptoms..⁹⁴ PA (b)(6),(b)(7)(C),(b)(7)(F) acknowledged attempts to check DANIEL's blood pressure ceased going forward because of DANIEL's continued refusals..⁹⁵

August 9, 2022

PA (b)(6),(b)(7)(C),(b)(7)(F) assessed DANIEL pursuant to a sick call request for an elastic wrap for ankle swelling..⁹⁶ PA (b)(6),(b)(7)(C),(b)(7)(F) informed DANIEL he would have to be seen in the medical clinic, to which DANIEL agreed and PA Robinson scheduled DANIEL for an appointment at the medical clinic for the next day..⁹⁷

August 10, 2022

PA (b)(6),(b)(7)(C),(b)(7)(F) evaluated DANIEL in the medical clinic, noting the following..⁹⁸

- DANIEL presented with high blood pressure at 163/99..⁹⁹ while all other vital signs were within normal limits;
- DANIEL reported swelling of his left ankle and foot for the past month with no related history of trauma or injury; and

⁸⁸ See clinical encounter titled: Severe Delusions, dated July 19, 2021.

⁸⁹ *Ibid.*

⁹⁰ ERAU interview with Dr. (b)(6),(b)(7)(C),(b)(7)(F) on April 3, 2024.

⁹¹ Blood pressure of 186/112 falls within AHA parameters for Stage 2 hypertension.

⁹² See encounter titled: Vitals, dated March 3, 2022.

⁹³ ERAU interview with PA (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

⁹⁴ PA (b)(6),(b)(7)(C),(b)(7)(F) provided ERAU with citations to research that indicates hypertensive urgency, in the absence of target end organ damage, has not been linked to increased morbidity or mortality. See (b)(6),(b)(7)(C),(b)(7)(F) email, dated April 3, 2024.

⁹⁵ See IHSC Refusal Form, dated March 3, 2022.

⁹⁶ See ICE Health Service Corp Sick Call Request Form, dated August 8, 2022.

⁹⁷ See encounter titled: Requesting ankle wraps, dated August 9, 2022.

⁹⁸ See encounter titled: LE edema, dated August 10, 2022.

⁹⁹ Blood pressure of 163/99 falls within AHA parameters for Stage 2 hypertension.

- DANIEL denied any pain or tingling but stated when he is attacked by electronic waves at night, he wraps his knee, and the pain goes away;
- DANIEL had a mass on his neck that protruded 2 inches by 2.5 inches and was anterior to his larynx and trachea;
- DANIEL reported he first noticed the mass in September 2021, as a small lump and it continued to grow; and
- DANIEL denied pain in his neck, fever, shortness of breath, or difficulty swallowing.

PA (b)(6),(b)(7)(C),(b)(7)(F) noted DANIEL's left leg was mildly larger when compared to his right leg and had full range of motion. PA (b)(6),(b)(7)(C),(b)(7)(F) recommended medications to treat DANIEL's hypertension, which DANIEL refused. PA (b)(6),(b)(7)(C),(b)(7)(F) submitted a referral for an ultrasound¹⁰⁰ of the neck mass and ordered a comprehensive metabolic panel¹⁰¹ and complete blood count.¹⁰²

August 11, 2022

Dr. (b)(6),(b)(7)(C),(b)(7)(F) evaluated DANIEL's neck mass and noted DANIEL reported that he treated the mass by gargling with soap and salt water.¹⁰³ DANIEL refused to provide a blood sample for the labs PA (b)(6),(b)(7)(C),(b)(7)(F) ordered but agreed to an ultrasound of the neck mass. Dr. (b)(6),(b)(7)(C),(b)(7)(F) ordered the ultrasound and scheduled DANIEL for a follow-up once the ultrasound was completed.

August 15, 2022

PA (b)(6),(b)(7)(C),(b)(7)(F) evaluated DANIEL and noted he complained about increased swelling in his right leg above the knee.¹⁰⁴ PA (b)(6),(b)(7)(C),(b)(7)(F) noted edema in both extremities, tenderness in the back of DANIEL's left knee with no mass felt, and increased fluid above the left kneecap. PA (b)(6),(b)(7)(C),(b)(7)(F) ordered tests for prostate-specific antigen,¹⁰⁵ thyroid stimulating hormone,¹⁰⁶ and a comprehensive metabolic panel. PA (b)(6),(b)(7)(C),(b)(7)(F) provided DANIEL with an elastic bandage and knee sleeve and completed a referral for an ultrasound of his left leg¹⁰⁷ to look for a deep vein thrombosis.¹⁰⁸

August 16, 2022

Dr. (b)(6),(b)(7)(C),(b)(7)(F) emailed (b)(6),(b)(7)(C),(b)(7)(F), Health Services Administrator (HSA), to follow-up on Dr. (b)(6),(b)(7)(C),(b)(7)(F) July 2021 recommendation to explore the possibility of DANIEL's transfer to

¹⁰⁰ An ultrasound is a noninvasive procedure that uses high-frequency sound waves to produce images of structures within the body.

¹⁰¹ A comprehensive metabolic panel is a group of 14 tests that measures substances in the blood to determine fluid balance, electrolyte levels, and level of functioning of the kidneys and liver.

¹⁰² A complete blood count evaluates the overall health of the body and detects a wide range of disorders, including anemia, infection, inflammation, bleeding disorders, or cancer.

¹⁰³ See Encounter titled: Neck Mass, dated August 11, 2022.

¹⁰⁴ See Encounter titled: Multiple complaints, dated August 15, 2022.

¹⁰⁵ A prostate-specific antigen test is a blood test used to screen for prostate cancer.

¹⁰⁶ A thyroid stimulating hormone test measures the amount of TSH in the blood.

¹⁰⁷ See Referral and Consultation Request and Hospital Transfer form, dated August 16, 2022.

¹⁰⁸ Deep vein thrombosis is a blood clot in a deep vein, most commonly in the legs or pelvis. See Referral and Consultation Request and Hospital Transfer form, dated August 15, 2022.

another ICE facility..¹⁰⁹ During an interview with ERAU, HSA (b)(6),(b)(7)(C),(b)(7)(F) stated she tried to find an alternate facility but was unsuccessful due to DANIEL's delusional diagnosis..¹¹⁰

August 17, 2022

Dr. (b)(6),(b)(7)(C),(b)(7)(F) conducted a follow-up with DANIEL, noting the following..¹¹¹

- DANIEL presented with high blood pressure at 168/90..¹¹² while all other vital signs were within normal limits;
- DANIEL complained of draining in his right ear with intermittent tinnitus..¹¹³ and
- DANIEL denied any pain, fever or chills, shortness of breath, or cough.

Dr. (b)(6),(b)(7)(C),(b)(7)(F) examination of DANIEL's ear showed it was blocked with clear discharge from the middle ear. Dr. (b)(6),(b)(7)(C),(b)(7)(F) diagnosed DANIEL with otitis media..¹¹⁴ and prescribed neomycin-polymyxin-hydrocortisone (HC)..¹¹⁵ 4 drops, twice daily, for 7 days. Dr. (b)(6),(b)(7)(C),(b)(7)(F) reviewed the ultrasound results of DANIEL's leg and neck and noted the ultrasound of his leg showed no evidence of deep vein thrombosis, but the ultrasound of his neck identified a thyroglossal duct cyst..¹¹⁶ and recommended a contrast-enhanced computed tomography (CT) scan..¹¹⁷ for further evaluation of the cyst..¹¹⁸ Dr. (b)(6),(b)(7)(C),(b)(7)(F) diagnosed DANIEL with thyroglossal cyst and uncontrolled blood pressure and ordered a prothrombin test..¹¹⁹ DANIEL was cooperative with medical staff and allowed his blood to be drawn for the labs ordered by Dr. (b)(6),(b)(7)(C),(b)(7)(F) and PA (b)(6),(b)(7)(C),(b)(7)(F) on August 10, 2022..¹²⁰ During an interview with ERAU, when asked about DANIEL's consistently elevated blood pressure readings, Dr. (b)(6),(b)(7)(C),(b)(7)(F) acknowledged his readings were high, but noted DANIEL did not have any symptoms associated with high blood pressure..¹²¹

¹⁰⁹ See Email from Dr. (b)(6),(b)(7)(C),(b)(7)(F) dated August 16, 2022.

¹¹⁰ ERAU interview with HSA (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

¹¹¹ See encounter titled: Neck Mass f/u, dated August 17, 2022.

¹¹² Blood pressure of 168/90 falls within AHA parameters for Stage 2 hypertension.

¹¹³ Tinnitus is the ringing of one's ear(s).

¹¹⁴ Infected otitis media is a bacterial infection of the middle ear.

¹¹⁵ Neomycin-polymyxin-hydrocortisone is a medication used to treat infections of the ear canal and to help provide relief from redness, irritation, and discomfort of certain ear problems.

¹¹⁶ Thyroglossal duct cysts are rare cysts in the throat that are present at birth and are almost always benign. These cysts are usually found and treated while people are age 10 and younger. Rarely, some people discover their cysts as adults.

¹¹⁷ A contrast-enhanced computed tomography (CT) scan refers to a technique used during a CT scan whereby a contrast agent is administered to the patient which allows certain structures in the body to become more clearly visible on CT images. See TridentCare Diagnostic Laboratories Report, dated August 15, 2022.

¹¹⁸ See TridentCare Diagnostic Laboratories Report, dated August 15, 2022.

¹¹⁹ A prothrombin test measures how quickly one's blood sample clots and can assist medical professionals in determining if one has a bleeding problem or if their+ blood does not clot properly.

¹²⁰ See encounter titled Prolactin, dated August 17, 2022. See also encounter titled: LE edema, dated August 10, 2022.

¹²¹ ERAU interview with Dr. (b)(6),(b)(7)(C),(b)(7)(F) on March 29, 2024.

August 25, 2022

PA (b)(6),(b)(7)(C),(b)(7)(F) met with DANIEL to review his lab results and noted DANIEL presented with high blood pressure at 162/86,¹²² while all other vital signs were within normal limits.¹²³ DANIEL's lab results were within normal limits except for the following:¹²⁴

- Elevated creatinine at 1.41 mg/dL;¹²⁵
- Elevated prolactin at 18.2 ng/mL;¹²⁶
- High prostate-specific antigens at 4.4 ng/mL;¹²⁷ and
- Low estimated glomerular filtration rate at 57 ng/mL.¹²⁸

PA (b)(6),(b)(7)(C),(b)(7)(F) discussed the lab results with DANIEL and educated him on the findings, advising further screening for prostate cancer. DANIEL denied the findings but agreed to additional labs in 3 to 4 months.

August 26, 2022

DANIEL was transported to the St. Joseph Medical Center (SJMC) for a CT scan of the mass on his neck.¹²⁹ SJMC medical staff conducted a CT scan without contrast because DANIEL refused the intravenous contrast-enhanced fluid required for the CT scan.¹³⁰ The CT scan identified a smooth cystic lesion¹³¹ to the left of midline of DANIEL's neck, measuring approximately 6 centimeters (cm) in width. DANIEL was cleared to return to NWIPC and was transported back to the facility by NWIPC security staff.

September 1, 2022

PA (b)(6),(b)(7)(C),(b)(7)(F) met with DANIEL to discuss findings of his CT scan, documenting the examination findings were normal except DANIEL had localized swelling of the mass on his neck and had an ear infection.¹³² PA (b)(6),(b)(7)(C),(b)(7)(F) prescribed ofloxacin,¹³³ 0.3 percent, 5 drops, 3 times a day, for 14 days and discussed a surgical consultation to remove the neck mass. DANIEL declined, stating he wanted to complete the antibiotic treatment because he believed the ear infection and neck mass

¹²² Blood pressure of 162/86 falls within AHA parameters for Stage 2 hypertension.

¹²³ See encounter titled: Lab review/Opt Out Testing Results, dated August 25, 2022.

¹²⁴ See LabCorp laboratory results, dated August 17, 2022.

¹²⁵ A creatinine test is a measure of how well the kidneys are functioning. Normal creatine levels are between 0.76 and 1.27 mg/dL.

¹²⁶ A prolactin test is a measure of how much hormone prolactin is found in the blood. Normal prolactin levels are between 4.0 and 15.2 ng/mL.

¹²⁷ A prostate-specific antigen test is a blood test used to screen for prostate cancer. Normal prostate-specific antigen levels are between 0 and 4 ng/mL.

¹²⁸ An estimated glomerular filtration rate test shows how well the kidneys are filtering. A normal estimated glomerular filtration rate is >59 (mL/min/1.73m2).

¹²⁹ See encounter titled Radiology appointment at St. Joseph Outpatient on 8/26/2022 at 1430 for CT neck English Speaking, dated August 26, 2022.

¹³⁰ See St. Joseph Medical Center Imaging Result, dated August 30, 2022.

¹³¹ A smooth cystic lesion is a sac that may be filled with air, fluid, or other material.

¹³² See encounter titled CT results, dated September 1, 2022.

¹³³ Ofloxacin is an antibiotic that treats infections caused by bacteria.

were related and caused by electronic wave torture and attacks by the government. PA (b)(6),(b)(7)(C),(b)(7)(F) agreed to revisit the surgical consult and would follow-up after the antibiotic treatment.

September 14, 2022

Dr. (b)(6),(b)(7)(C),(b)(7)(F) submitted a referral for surgical consultation with an otolaryngology.¹³⁴ specialist for DANIEL's neck mass.¹³⁵ DANIEL refused the otolaryngology consultation, noting on the refusal form he believed the consultation was "premature surgery misunderstanding."¹³⁶

September 22, 2022

PA (b)(6),(b)(7)(C),(b)(7)(F) conducted a follow-up with DANIEL to assess his ear infection.¹³⁷ DANIEL had completed the course of antibiotics and denied ear pain and improved hearing. PA (b)(6),(b)(7)(C),(b)(7)(F) recommended continued monitoring and instructed DANIEL to request medical if his symptoms returned.

September 2022 – August 2023

NWIPC medical and mental health care staff continued their attempts to treat DANIEL for his mental health and medical issues, DANIEL often refused care, including annual physical examination, blood draws for lab tests, hypertension chronic care, and provider appointments. Staff were unable to record any blood pressure checks in 2023 as DANIEL continued to refuse blood pressure checks.¹³⁸

August 15, 2023

Dr. (b)(6),(b)(7)(C),(b)(7)(F) changed DANIEL's status to a detainee with serious mental illness (SMI) after discussion with IHSC headquarters personnel.¹³⁹ Dr. (b)(6),(b)(7)(C),(b)(7)(F) placed a global alert.¹⁴⁰ for DANIEL's status change and instituted daily checks by mental health staff per IHSC policy.¹⁴¹

August 2023 – March 2024

NWIPC medical and mental health care staff continued their attempts to treat DANIEL for his mental health and medical issues, DANIEL continued to refuse care, including his annual physical examination, blood draws for lab tests, and provider appointments. Staff were unable to

¹³⁴ Otolaryngology is a medical specialty for the study of ear, nose, and throat.

¹³⁵ See ICE Referral Consultation Request and Hospital Transfer Form, dated September 14, 2022.

¹³⁶ See IHSC Refusal Form, dated September 14, 2022.

¹³⁷ See encounter titled F/u ear infection, dated September 22, 2022.

¹³⁸ NWIPC staff obtained signed refusals for encounters that DANIEL refused.

¹³⁹ ERAU interview with Dr. (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024.

¹⁴⁰ A global alert ensures providers who access a patient's medical record are aware the patient has been diagnosed with an SMI.

¹⁴¹ See IHSC Directive: 07-05, Serious Mental Disorders and Conditions, dated July 25, 2019.

Daily checks of DANIEL were conducted as required by SMI policy.

record any blood pressure checks in 2024 as DANIEL continued to refuse blood pressure checks..¹⁴²

Dr. [REDACTED] and Dr. [REDACTED] pursued DANIEL's transfer to the Krome North Service Processing Center (KNSPC)..¹⁴³ On January 10, 2024, Dr. [REDACTED] informed Dr. [REDACTED] that after discussion with the IHSC Western Regional Clinical Director, it was determined there was no medical basis for transferring DANIEL to KNSPC and administrative transfer would need to be pursued..¹⁴⁴ In an email to Dr. [REDACTED] on January 16, 2024, [REDACTED] KNSPC clinical director, stated that since DANIEL did not have medical conditions meriting a medical transfer, there was no need for transfer to a Level-4 facility; further, that if he were to be transferred to KNSPC for administrative reasons, he would be evaluated upon arrival like any other patient..¹⁴⁵

March 5, 2024

During daily SMI rounds, Dr. [REDACTED] noted he observed DANIEL in his cell, relaxing on his bed. DANIEL remarked he was having a good day but expressed concern about his throat because when he sings, all the poison comes up in his throat and makes him feel hoarse..¹⁴⁶ Dr. [REDACTED] offered referral to a medical provider, but DANIEL declined.

March 7, 2024 - Day of Death

RHU DOs conducted required health and welfare checks of DANIEL at least every 30 minutes on an irregular basis throughout the morning noting no significant concerns.

At 7:46 a.m., [REDACTED], RN, conducted nursing rounds in the RHU..¹⁴⁷ noting DANIEL did not report any medical, dental, or mental health needs..¹⁴⁸ During an interview with ERAU, RN [REDACTED] stated DANIEL was awake and sitting at his sink and was washing his face..¹⁴⁹ RN [REDACTED] reported she said good morning to DANIEL, and he responded to her by stating, "Good morning."

At 8:44 a.m., [REDACTED], NWIPC DO, conducted a health and welfare check of DANIEL and observed DANIEL was leaning over his toilet and there was vomit in the sink..¹⁵⁰ DANIEL reported he wasn't feeling well but did not need medical help. DO [REDACTED] informed [REDACTED] [REDACTED], NWIPC DO, who called the medical department to speak with PA [REDACTED]..¹⁵¹

¹⁴² NWIPC staff obtained signed refusals forms for encounters DANIEL that refused.

¹⁴³ See emails dated January 5 – 16, 2024.

¹⁴⁴ See [REDACTED] email, dated January 10, 2024.

¹⁴⁵ See [REDACTED] email, dated January 16, 2024.

¹⁴⁶ See encounter titled: Daily SMI, dated March 5, 2024.

¹⁴⁷ See RHU fixed camera video.

¹⁴⁸ See Encounter titled SEG ROUNDS, dated March 7, 2024.

¹⁴⁹ ERAU interview with RN [REDACTED] on April 4, 2024.

¹⁵⁰ ERAU interview with DO [REDACTED] on March 29, 2024.

See Memorandum by DO [REDACTED] dated March 7, 2024.

¹⁵¹ ERAU interview with DOs [REDACTED] on April 2, 2024.

See Memorandum by DO [REDACTED] dated March 7, 2024.

(b)(6),(b)(7)(C),(b)(7)(F) NWIPC DO, was assigned to the medical post, and advised DO (b)(6),(b)(7)(C),(b)(7)(F) that PA (b)(6),(b)(7)(C),(b)(7)(F) was with a patient and would call her back.¹⁵²

At 8:50 a.m., PA (b)(6),(b)(7)(C),(b)(7)(F) returned DO (b)(6),(b)(7)(C),(b)(7)(F) call and DO (b)(6),(b)(7)(C),(b)(7)(F) described DANIEL's symptoms.¹⁵³ PA (b)(6),(b)(7)(C),(b)(7)(F) advised DO (b)(6),(b)(7)(C),(b)(7)(F) to call Information Management System (IMS).¹⁵⁴ if she felt the condition was urgent because even if he reported to the RHU, he could not treat DANIEL in the RHU and based on his experience, DANIEL would refuse to come to medical. After speaking with PA (b)(6),(b)(7)(C),(b)(7)(F), DO (b)(6),(b)(7)(C),(b)(7)(F) went to DANIEL's cell to check on him.¹⁵⁵ DANIEL told DO (b)(6),(b)(7)(C),(b)(7)(F) that he ate some pudding the previous night and vomited in his sink. DANIEL refused to go to medical and stated he was fine after vomiting the pudding.

At 8:54 a.m., (b)(6),(b)(7)(C),(b)(7)(F) NWIPC captain, was conducting rounds in the RHU and noted she observed DANIEL in his cell, sitting on his bunk with green underwear on his head, and that he smiled and waved to her.¹⁵⁶

At 9:09 a.m., (b)(6),(b)(7)(C),(b)(7)(F) NWIPC DO, conducted a health and welfare check and noted he was sitting upright, and in no apparent distress.¹⁵⁷

At 10:22 a.m., (b)(6),(b)(7)(C),(b)(7)(F) LCSW, conducted an SMI encounter with DANIEL and noted he appeared well-groomed and had a pleasant demeanor, stating his was resting because they poisoned his pudding.¹⁵⁸ During an interview with ERAU, LCSW (b)(6),(b)(7)(C),(b)(7)(F) stated DANIEL appeared the same as any other day and showed no emotional or physical distress.¹⁵⁹

At 10:46 a.m., DO (b)(6),(b)(7)(C),(b)(7)(F) conducted a health and welfare check of DANIEL and observed him slumped over on his bed.¹⁶⁰ DO (b)(6),(b)(7)(C),(b)(7)(F) called to DANIEL and knocked on his cell door, but DANIEL did not respond. DO (b)(6),(b)(7)(C),(b)(7)(F) yelled for DO (b)(6),(b)(7)(C),(b)(7)(F) who announced an IMS over the radio and asked the control center officer to open DANIEL's cell door.¹⁶¹ DO (b)(6),(b)(7)(C),(b)(7)(F) and (b)(6),(b)(7)(C),(b)(7)(F) entered DANIEL's cell and DO (b)(6),(b)(7)(C),(b)(7)(F) performed a sternum rub.¹⁶² and tried to get a pulse but was unsuccessful.¹⁶³

¹⁵² ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024. See Memorandum by DO (b)(6),(b)(7)(C),(b)(7)(F) dated March 7, 2024.

¹⁵³ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) on April 2, 2024. See Memorandum by DO (b)(6),(b)(7)(C),(b)(7)(F) dated March 7, 2024.

¹⁵⁴ IMS is the term used for any emergent need for medical care.

¹⁵⁵ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) April 2, 2024. See Memorandum by DO (b)(6),(b)(7)(C),(b)(7)(F) dated March 7, 2024.

¹⁵⁶ ERAU interview with Captain (b)(6),(b)(7)(C),(b)(7)(F) March 29, 2024. See Video surveillance footage, dated March 7, 2024.

¹⁵⁷ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) April 4, 2024.

¹⁵⁸ See encounter titled request Daily SMI, dated March 7, 2024.

¹⁵⁹ ERAU interview with LCSW (b)(6),(b)(7)(C),(b)(7)(F) April 4, 2024.

¹⁶⁰ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) March 29, 2024.

¹⁶¹ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) April 2, 2024.

¹⁶² A sternum rub is application of painful stimulus with the knuckles of a closed fist to the center of the chest of a patient who is not alert and does not respond to verbal stimuli.

¹⁶³ ERAU interview with DO (b)(6),(b)(7)(C),(b)(7)(F) April 2, 2024.

At 10:48 a.m., DOs [REDACTED] arrived at DANIEL's cell.¹⁶⁴ DOs [REDACTED] moved DANIEL off the bed, onto the floor with DO [REDACTED] holding DANIEL's legs and DO [REDACTED] supporting his head.

At 10:49 a.m., LTs [REDACTED] and RN [REDACTED] arrived at DANIEL's cell.¹⁶⁵

At 10:50 a.m., PA [REDACTED] arrived on scene with an emergency bag, followed by LPN [REDACTED] and RN [REDACTED] with a gurney, oxygen tank, and automated external defibrillator (AED).¹⁶⁶ DO [REDACTED] said he felt a pulse, but DANIEL was not breathing. PA [REDACTED] pulled DANIEL out of his cell into the hallway immediately outside the cell, instructed [REDACTED] to call for emergency medical services (EMS), and began performing chest compressions.¹⁶⁷ [REDACTED] ordered the control center officer to call 911, who called the Tacoma Fire Department and requested EMS for DANIEL.

At 10:51 a.m., RN [REDACTED] placed an Ambu bag¹⁶⁸ over DANIEL's mouth and attempted to push air into DANIEL but was unable to do so. PA [REDACTED] and RN [REDACTED] attempted to reposition DANIEL's head to open his airway but were unsuccessful.¹⁶⁹

At 10:54 a.m., LPN [REDACTED] placed the AED pads on DANIEL, which advised and administered a shock. Following the shock, DO [REDACTED] resumed chest compressions.

At 10:55 a.m., RN [REDACTED] took over chest compressions while PA [REDACTED] and RN [REDACTED] continued to work on opening DANIEL's airway.¹⁷⁰ The AED directed rescue breaths, but PA [REDACTED] and RN [REDACTED] were unable to get air into DANIEL's lungs.

At 10:56 a.m., RN [REDACTED] remarked she did not see anything in his mouth, presumably referring to an object that may be obstructing the airway. During an interview with ERAU, PA [REDACTED] stated the mass on DANIEL's neck inhibited establishment of an airway.¹⁷¹

At 10:58 a.m., EMS arrived in the RHU and assumed care of DANIEL.¹⁷² PA [REDACTED] informed the paramedics that DANIEL had a mass on his throat and that may be obstructing the airway.

At 10:59 a.m., paramedics attempted to intubate DANIEL but were initially unsuccessful.¹⁷³ Paramedics continued to move the position of DANIEL's head and were able to introduce air by lifting DANIEL's head and tilting it to the left.

¹⁶⁴ See Video surveillance footage, dated March 7, 2024.

¹⁶⁵ See Video surveillance footage, dated March 7, 2024.

¹⁶⁶ Automated external defibrillator (AED) is a medical device that analyzes the heart's rhythm. If necessary, it delivers an electrical shock, known as defibrillation, which helps the heart re-establish an effective rhythm.

¹⁶⁷ See Video surveillance footage, dated March 7, 2024.

¹⁶⁸ Ambu bag is a medical tool which forces air into the lungs of patient who are not breathing.

¹⁶⁹ See Video surveillance footage, dated March 7, 2024.

¹⁷⁰ *Ibid.*

¹⁷¹ ERAU interview with PA [REDACTED] April 2, 2024.

¹⁷² See Video surveillance footage, dated March 7, 2024.

¹⁷³ See Prehospital Care Report, dated March 7, 2024.

At 11:41 a.m., EMS was instructed by Dr. (b)(6),(b)(7)(C),(b)(7)(F) at Good Samaritan Hospital to terminate lifesaving efforts, and DANIEL was pronounced deceased..¹⁷⁴

Post-Death Events

On March 7, 2024, Supervisory Detention and Deportation Officer (b)(6),(b)(7)(C),(b)(7)(F) ERO Seattle, notified the Trinidad and Tobago consulate of DANIEL's death..¹⁷⁵

On March 13, 2024, (b)(6),(b)(7)(C),(b)(7)(F) Assistant Field Office Director, ERO Seattle, sent DANIEL's next of a condolence letter..¹⁷⁶

FINDINGS

ERAU reviewed the medical care NWIPC provided DANIEL, as well as the staff's efforts to ensure his safety and security while detained at the facility. ERAU found NWIPC failed to comply with 1 requirement of the ICE 2011 PBNDS (Revised) 2016.

1. *Medical Care*, Section (AA)(4), which states "Prior to administration of psychotropic medications, a separate documented informed consent, that includes a description of the medication's side effects, shall be obtained."
 - On April 3, 2020, PA (b)(6),(b)(7)(C),(b)(7)(F) did not obtain DANIEL's consent for fluoxetine, a psychotropic medication. Furthermore, PA (b)(6),(b)(7)(C),(b)(7)(F) did not document that they discussed the medication's side effects.

EXHIBITS

1. State of Washington Certificate of Death, issued June 26, 2024.
2. Pierce County Medical Examiner's Office Autopsy Report, issued June 3, 2024.

¹⁷⁴ See Prehospital Care Report, dated March 7, 2024.

¹⁷⁵ See Trinidad and Tobago death notification e-mail, dated March 7, 2024.

¹⁷⁶ See Condolence Letter to Next of Kin, dated March 13, 2024. DANIEL's next of kin declined ERO Seattle's attempts to return his property to the next of kin.